

Capacity & Scheduling

What we are looking to build. Three connected capabilities decide whether a referral becomes a delivered visit — and a platform has to serve all three.

THE ENVELOPE

Capacity Management

How much work a branch can deliver, and how much room is left.

Workforce supply

Who we have, and what each of them is qualified to do.

- Roster by discipline, role (assessing vs. assistant), FTE and employment type
- Start-of-care capable clinicians, measured as their own distinct capacity
- Specialty competencies, plus orientation and ramp status
- Per-diem and float pool
- On-call and weekend rotation load

Availability & reach

When each clinician works, and where they can actually get to.

- Approved time off and working availability
- Clinician territory assignment by zip, against branch coverage area
- Reachability from the clinician's home base, measured by drive time

The capacity math

What is committed, what is open, and what is arriving.

- Visit weighting — the point value per visit type, and the productivity target and ceiling it is measured against
- Committed load vs. open room, by day, week, discipline and territory
- Referral inflow and discharge outflow against the envelope

COORDINATION

FILLING THE ENVELOPE

Scheduling Engine

Which clinician, which day, which route.

Demand

Everything the engine must know about what has been ordered.

- Plan-of-care orders, ordered frequency and visit types, from the EMR (Home Care Home Base) and the intake platform
- Authorization status and payer rules attached to each visit
- Order, consent and readiness state — what is schedulable, not merely ordered
- The compliance window each visit has to fall inside

Matching

Putting the right clinician with the right patient.

- Discipline and role match, specialty competency, acuity to skill level
- Clinician working pattern, needs and restrictions
- Patient and caregiver needs, restrictions and availability, sourced from the patient
- Clinical timing — diagnosis-driven cadence, competing appointments, infection-control sequencing
- Continuity of care, and supervisory visit dependencies

Routing & the week

A day that works, inside a week that holds.

- Home base start and end, route proximity, mileage
- Appointment time windows and intra-day sequencing
- Front-loading, pace against the plan of care, day-by-day balancing

Exceptions

What happens when the plan breaks.

- Missed-visit management and tracking — and reducing the occurrence
- Reassignment, coverage, and rebooking inside the window

MAKING IT HAPPEN

Engagement

Turning a schedule into delivered visits — with patients, clinicians and the office.

Before the visit

Securing the visit before the clinician drives.

- New-patient welcome call
- Patient availability captured before the visit is booked
- Automated reminders, day-before confirmation, en-route notification
- Automating the day-before outreach clinicians perform manually today, for every patient, every day
- Channel and communication-preference management

When plans change

Recovering a visit rather than losing it — with the patient and the clinician.

- Reschedule coordination with the patient
- Coverage coordination — matching potential clinicians to an open need, and reaching them directly
- Call-out coverage, and the urgent or prioritized needs that surface during the day
- Failed-visit and no-show follow-up and rebooking

Across the care team

Keeping clinicians and the office on the same schedule.

- Multi-discipline visit coordination
- Care-team and office coordination updates
- The staff time coordination consumes today